INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Eribulin · Days 1 and 15 of 28

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00743
Version 3

> Eribulin on Days 1 and 15 of each 28-day cycle. Days 8 and 22 are planned treatment-free weeks. This alternative schedule requires a specific discussion and consent.

Treatment stages

01 **Eribulin · Days 1 and 15 of 28**
28-day cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. New chest pain, fainting, severe breathlessness, fever or sudden illness needs urgent assessment. Report worsening numbness, severe constipation, mouth ulcers or inability to drink promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your medicines and what they do

Control advanced cancer and symptoms while balancing side effects and treatment burden.

CHEMOTHERAPY

Eribulin

Halaven · eribulin base



Stops microtubules growing

Eribulin blocks growth of the tiny structures cells need to divide. It can slow cancer growth, but can also affect blood cells and nerves.

Cell-division structures → Growth blocked → Cell division disrupted

What to watch for

Report numbness, weakness or difficulty with walking or hand tasks before the next dose. Fever or sudden illness needs immediate advice; palpitations or fainting need urgent assessment.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

This fortnightly eribulin schedule is an option for eligible previously treated advanced breast cancer. It adapts the licensed visit pattern, so the reason for the alternative schedule, uncertainty and off-label consent need explicit discussion.

Your treatment setting

Locally advanced or metastatic breast cancer after prior chemotherapy · alternative eribulin schedule

Your exact schedule

Eribulin on Days 1 and 15 of each 28-day cycle. Days 8 and 22 are planned treatment-free weeks. This alternative schedule requires a specific discussion and consent.

Alternatives

Discuss biomarker-directed, endocrine, chemotherapy and supportive-care options appropriate to this setting, including the option not to proceed.

Why this treatment may be chosen

Discuss how this evidence and the available alternatives apply to your own situation.

CHAPTER 02

Your schedule, tests and appointments

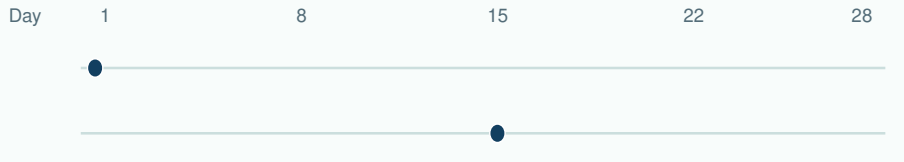
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment and review

Eribulin · Days 1 and 15 of 28

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

28-DAY CYCLE

Eribulin · Days 1 and 15 of 28

Repeat according to benefit and tolerance

Eribulin on Days 1 and 15 of each 28-day cycle. Days 8 and 22 are planned treatment-free weeks. This alternative schedule requires a specific discussion and consent.

Your team confirms the active medicines, treatment days, dose changes and next review.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm completion.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00743 v3 · official regimen · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · 2026 · EMA Halaven product information · blood counts before every dose

THE WHOLE TREATMENT JOURNEY

Eribulin · Days 1 and 15 of 28: treatment and checks

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Treatment plan NCCP 00743	Confirm alternate-schedule consent	Day 1 infusion over 5 minutes	Day 15 infusion; q28-day course	Until progression / intolerance	Next treatment plan
Blood / organ function NCCP 00743	full blood count (FBC), renal, liver baseline	FBC before dose (EMA)	FBC before every dose; renal/liver qcycle	Review count recovery / reductions	Investigate persistent abnormalities

Care / check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Heart / electrolytes NCCP 00743 · SMPC	Correct K, calcium and Mg	electrocardiogram (ECG) if cardiac / QT risk	Periodic electrolytes; ECG when indicated	Review interacting QT medicines	Report palpitations / persistent neuropathy
Cancer review METASTATIC · MARKERS	Confirm setting	Agree review plan	Response / symptom review	Next treatment / completion	Agreed surveillance

What each check is for

1 Before treatment

Before the first dose

Locally advanced or metastatic breast cancer after prior chemotherapy · alternative eribulin schedule. NCCP specifies FBC, kidney and liver tests at baseline and the start of each cycle. EMA Halaven information additionally requires FBC before every dose, including Day 15. Correct low potassium, calcium and magnesium before treatment; monitor periodically.

Ask / act: Confirm eligibility, test results and medicine list.

2 Other checks

At the specified review points

ECG monitoring is indicated with heart failure, bradyarrhythmia, QT-prolonging medicines or electrolyte disturbance. Review QT risk when choosing antiemetics; monitor neuropathy.

Ask / act: Confirm who arranges and reviews each check.

3 Support and practical plan

At each treatment or phase change

No routine hypersensitivity premedication is specified. Low-risk antiemetic support is reviewed separately, including QT interactions.

Ask / act: Choose the matching support plan; confirm active medicines.

4 Response and follow-up

Agreed review points

Agree the scan type and review interval for the known disease sites. Review response, symptoms and toxicity together; new or worsening symptoms may need an earlier assessment. Keep the agreed follow-up and report persistent or new symptoms between visits.

Ask / act: Review symptoms, results and patient priorities.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your treatment and access plan

STEP 1

Before treatment

FBC, kidney and liver tests at baseline and the start of each cycle. Confirm count recovery for each administration. Correct low potassium, calcium and magnesium before treatment and monitor periodically.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. A port is not automatically required; insertion and removal are separate procedures with individual instructions.

STEP 3

On the treatment day

Confirm the medicines due that day, any premedicines and the reviewed blood results. Tell staff immediately about pain, burning, rash or breathlessness.

STEP 4

After the visit

No routine take-home infusion pump is required for this regimen. Keep line-care, urgent-contact and next-review instructions.

Visit length

Eribulin is given over 5 minutes in this source. Preparation and blood review add to the visit; doses are on Days 1 and 15.

Line care

A PICC or port has its own care plan. Report fever, redness, swelling, pain, leakage or line damage; do not remove or adjust it yourself.

Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Support for this visit

No routine hypersensitivity premedication is specified. Low-risk antiemetic support is reviewed separately, including QT interactions.

Checks across treatment

Your team arranges tests and reviews the results before treatment decisions.

Blood and organ checks

NCCP specifies FBC, kidney and liver tests at baseline and the start of each cycle. EMA Halaven information additionally requires FBC before every dose, including Day 15. Correct low potassium, calcium and magnesium before treatment; monitor periodically.

Other treatment-specific checks

ECG monitoring is indicated with heart failure, bradyarrhythmia, QT-prolonging medicines or electrolyte disturbance. Review QT risk when choosing antiemetics; monitor neuropathy.

Cancer review and follow-up

Agree the scan type and review interval for the known disease sites. Review response, symptoms and toxicity together; new or worsening symptoms may need an earlier assessment.

Bone health

Bone-health assessment follows endocrine therapy, ovarian suppression, menopause and other fracture risks. It has its own monitoring plan.

The wider treatment plan

Review response, toxicity and priorities to decide whether to continue, change or stop. Supportive and palliative care can accompany anticancer treatment.

Medicines review

Check new prescriptions, over-the-counter medicines and supplements with the oncology pharmacist before taking them.

After treatment

Keep the agreed follow-up and report persistent or new symptoms between visits.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Eribulin · Days 1 and 15 of 28

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment · Day 1	 **Day 1** intravenous (IV)
Treatment · Day 15	 **Day 15** IV
Week without treatment	 **Days 8–14** No routine Day 8 dose
Week without treatment	 **Days 22–28** No routine Day 22 dose
Blood counts / infection	 **Days 1–28** Risk varies; fever needs prompt advice
Energy / appetite	 **Days 1–28** May vary and build across cycles
Numbness / tingling	 **Days 1–28** Can build across doses
Rhythm / fainting symptoms	 **Days 1–28** Report promptly whenever they occur

Timing varies: Symptoms can occur on any day, including days without treatment. The bands are orientation, not predicted symptom-free periods.

Treatment-day changes: A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Why these bands stay broad: Treatment markers show the planned days. Symptom bands show broad possibilities, not measured risk curves or guaranteed symptom-free days. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00743 v3 · official regimen ↗

There are no guaranteed "safe days". Fever, shivers or suddenly feeling unwell needs prompt advice on any day, including planned treatment-free weeks.

Between-treatment notes

Day 1–28: Reference cycle

Eribulin on Days 1 and 15 of each 28-day cycle. Days 8 and 22 are planned treatment-free weeks. This alternative schedule requires a specific discussion and consent. Follow your written appointment and medicine plan. A calendar does not authorise a dose.

Support around the medicines

1 **Blood counts and infection**

Fever, shivers or sudden illness needs prompt advice on any day.

2 **Nerve symptoms**

Report numbness, tingling or weakness before the next dose.

3 **Support medicines**

Follow the prescription for the active medicine and treatment day.

4 **Review the plan**

Discuss symptoms, recovery and the next treatment decision with the team.

Questions worth asking

Who arranges the checks and explains the results?

Agree the due window, booking team, result reviewer and contact route if a test or appointment is missing. This page does not record completed checks.

Try explaining the plan back: Which medicines and doses are prescribed, when are they due, which checks are needed, and what would make you call the team today?

Eribulin

- Low blood counts, infection and sepsis
- Numbness, tingling or weakness, sometimes lasting
- Fatigue, hair loss, nausea and constipation
- Changes to heart rhythm / QT in susceptible people

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Eribulin · Days 1 and 15 of 28 · each treatment day

Active medicines: Eribulin

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Each treatment day only Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- No routine hypersensitivity premedication is specified. Low-risk antiemetic support is reviewed separately, including QT interactions.
- This is not the licensed Day 1 / Day 8 schedule. The reason for the alternative schedule, evidence limits and off-label consent are part of the treatment discussion.
- Review interactions, QT risk and electrolytes before selecting an antiemetic; individualise rather than automatically issuing this draft.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Eribulin · Days 1 and 15 of 28

Eribulin on Days 1 and 15 of each 28-day cycle. Days 8 and 22 are planned treatment-free weeks. This alternative schedule requires a specific discussion and consent.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hands / feet

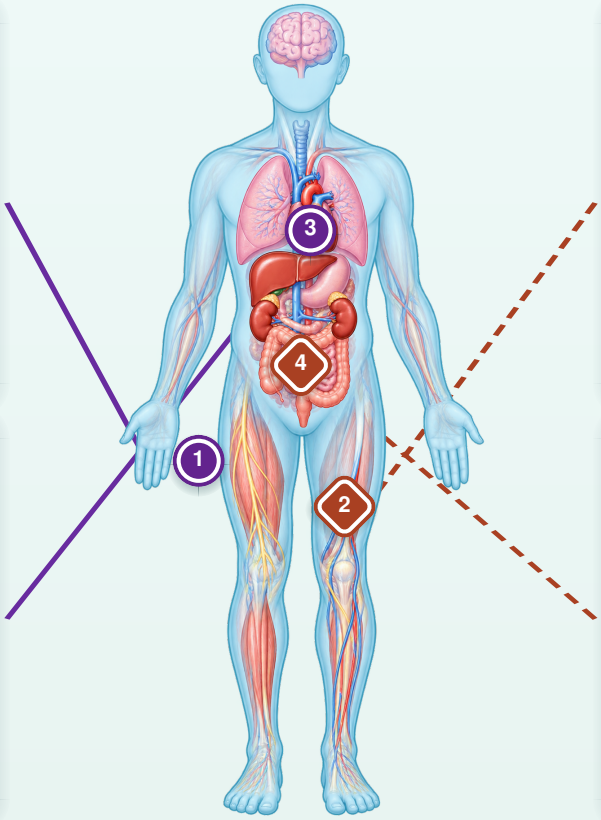
Eribulin

Report numbness, tingling, weakness or loss of function.

2 Blood / infection

Eribulin

Fever, shivers or suddenly unwell: call immediately.



3 Heart rhythm

Eribulin

Fainting or new palpitations need urgent assessment.

4 Bowel

Eribulin

Report persistent constipation, abdominal pain or vomiting.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Selected medicine-specific effects; this is not a complete list. Report new or persistent symptoms even after treatment stops.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00743 v3 · official regimen**
   https://healthservice.hse.ie/documents/6623/743_v3_eriBULin_Monotherapy_28_day.pdf
2. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
3. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
4. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
5. **EMA Halaven product information · blood counts before every dose**
   https://www.ema.europa.eu/en/documents/product-information/halaven-epar-product-information_en.pdf
6. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
7. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
8. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
9. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
10. **HSE · chemotherapy side effects and urgent advice**
    https://www2.hse.ie/conditions/chemotherapy/side-effects/
11. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
12. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf